

Basics of health Care

- **Course objective :** At the end
- Explain the health care system
 - Apply health ethics
 - Demonstrate client/patient approach
 - Demonstrate nutritional history taking and physical examination
 - Demonstrate vital sign measurement
 - List legislation and standards in infection prevention control
 - Explain principles of cleaning and ventilation
 - Demonstrate basic and selected common clinical procedures

Chapter 1: Definition of important terminologies,: Health, illness, disease

- ❖ Health: shows various and diverse meanings
- **Lay Point of view** : Persons are healthy when they are doing **their activities** with **no apparent symptoms** of disease in them.
- The oxford Dictionary describes health as ‘the state of being free from illness or injury’.

Definition important terminologies...

- **Professional points of view:** health is defined as s a measure of the state of the **physical bodily Organs**, and the ability of the body as a **whole to** function.
- It refers to **freedom** from **medically defined** diseases.

Definition important terminologies...

- **WHO definition:** world Health Organization (WHO) described health in 1948, in the preamble to its constitution,
 - “A state of **complete physical, mental, and social well-being** and **not merely** the absence of disease or infirmity”

Illness

- Is a feeling of **unhealthy** which is **entirely personal**, interior to the person.
- Often it accompanies disease, but the disease may be undeclared, as in the early stages of cancer or tuberculosis or diabetes.
- Sometimes illness exists where no disease can be found.
- It is more of a subjective feeling

Disease

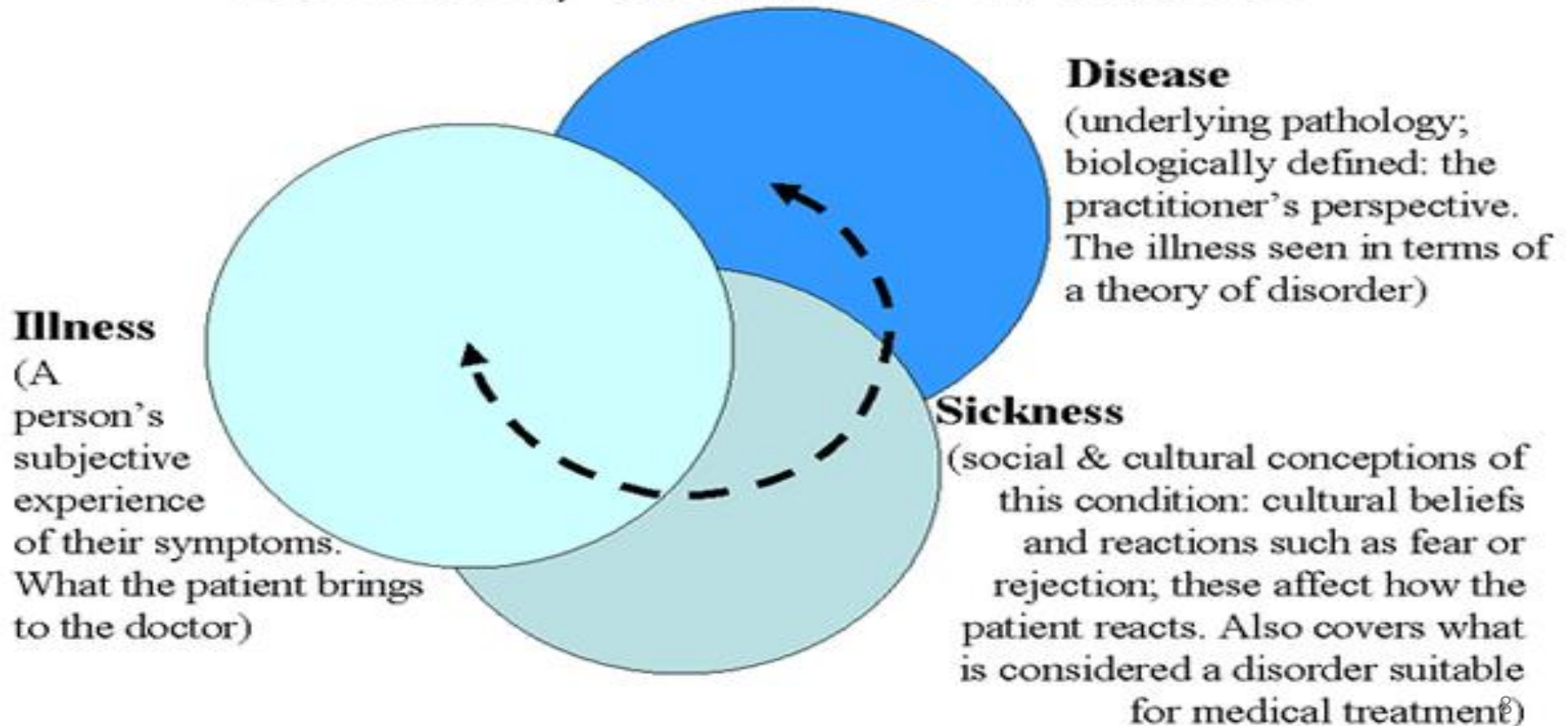
- is a **pathological** process, most often **physical** as in throat infection, or cancer of the bronchus, or hidden like mental illness
- The quality which identifies disease is some deviation from a biological norm.
- Disease then, is the pathological process, deviation from a biological norm
- It refers to a condition where the body or the parts of the body of a person does not work properly.
- There is usually a **pathological** reason behind the condition. ⁶

Sickness

- Is the role negotiated with **society**
- It is the external and public mode of **unhealthy**
- Chronic disease are much less secure than those with an acute one; those with a psychiatric disease than those with a surgical one

Different b/n Disease illness and Sickness

Three Perspectives: Disease, Sickness & Illness



Summary

1. illness is a **vague** condition that causes discomfort or pain.

- It is mostly curable.

2. disease refers to a condition that has established **reasons** behind it.

- There are some diseases that cannot be cured, only managed

Nutrition and health

INTRODUCTION

- Nutrition defined as the science of food and its relationship to health.
- The word nutrient or “**food factor**” is used for specific dietary constituents such as proteins, vitamins and minerals.
- Dietetics is the **practical application** of the principles of nutrition; it includes the **planning of meals** for the well and the sick.
- Good nutrition means “maintaining a nutritional status that enables us to grow well and enjoy good health.”

Energy requirement (ER)

- Amount of food energy needed to balance energy expenditure and intake
- Factors considered (body size and composition, physical activity, developmental stage, physiological changes (pregnancy lactation and Menstruation))
- For healthy, well-nourished adults, it is equivalent to total energy expenditure

Energy requirement ...

- Energy requirements also increase during illnesses like HIV.
- Refer table 1 for average daily energy requirements

Table 1: Average Daily Energy Requirements in Calories (*Source: WHO, 1993*)

	Status/Age	Energy Need (C)
Men	Average Active	2430
Women	Average active	2170
	Pregnant	2460
	Lactating	2570
Children	6–11 months old	730
	1–3 years old	1250
	2–5 years old	1500
	5–10 years old	1800

Calories requirements ...

Boys	10-14 years old	2360
	15-18 years old	2800
Girls	10-14 years old	2040
	15-18 years old	2100

- Micronutrient daily requirements are measured by recommended nutrient intake (RNI) which varies with age, sex and physiologic status.

**Table 2: Recommended daily
micronutrient intake (RNI) for selected
micronutrient**

Nutrient	Unit	RNI	Child 1 - 3 Yrs	RNI Child 4 – 6 Yrs	RNI Child 7 – 10 Yrs	RNI Female: 19 – 50Yrs	RNI Male: 19 -50Yrs	RNI Pregnant Women
Vitamin A	mg re	0.4000	0.4000	0.4500	0.5000	0.5000	0.6000	0.8000
Iron	mg	18.600 0	11.600 0	12.600 0	17.800 0	29.4000	13.7000	30.0000
Iodine	mg	0.0900	0.0900	0.0900	0.0900	0.1500	0.1500	0.2000

Nutrition And HIV

❖ Effects of malnutrition on HIV disease

- Malnutrition weakens the immune system and thus the ability of the individual to fight the infection
- Decreases CD4 count leads to appearance of opportunistic infections (OIs)
- Appearance of OIs leads to increase viral replication and further damage to the immune system
- Delays/weakens reaction to infections
- Malnutrition is associated with poor treatment outcome.
- Patient with SAM BMI < 16 are 4 to 5 times more likely to die in the first 90 days after starting ART.
- Patients with MAM are 2 to 3 times more likely to die in the first 90 days after starting ART

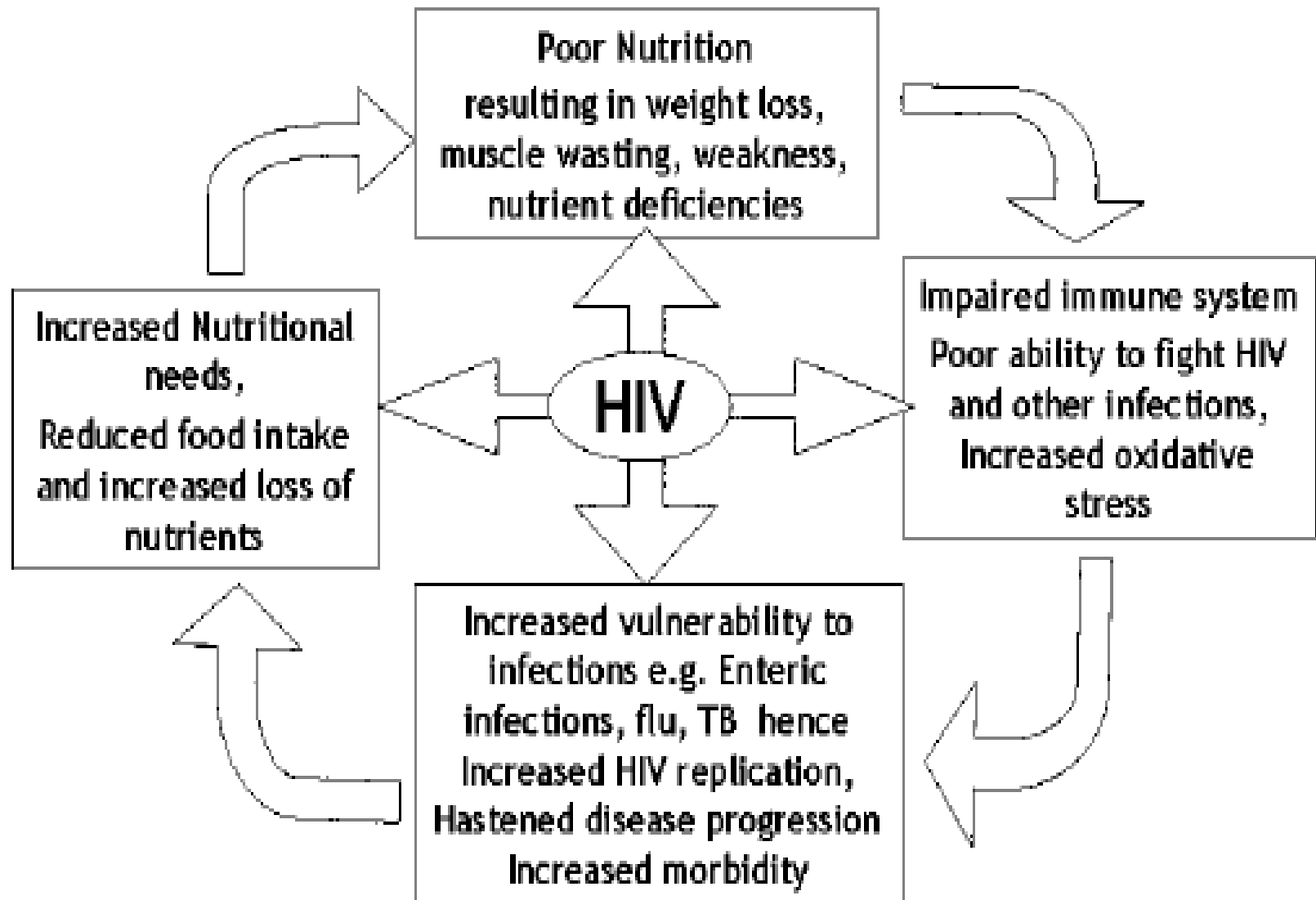
Effects of HIV on nutrition

- Decreased amount of food **consumption** (for example, due to mouth and throat sores, loss of appetite, side effects of medications, household food insecurity, etc.).
- Decreased nutrient absorption due to HIV infection of intestinal cells, frequent diarrhoea and vomiting, and opportunistic infections
- Increased energy requirement!!
 - asymptomatic adults requires 10-15 percent additional energy;
 - symptomatic adults require 20-30 percent additional energy

Effects of HIV on nutrition...

- HIV-positive children who are symptomatic but not losing weight require 20-30 percent additional energy;
- HIV-positive children who are symptomatic and losing weight require 50 – 100 percent additional energy.)
- Unlike past believes, evidences have shown that there is no **extra protein** and micronutrients needs above the RDA

Vicious Cycle of Malnutrition and HIV



Effects of good nutrition on HIV

- Decreases weight loss
- May delay disease progression and death
- Reduces incidences of opportunistic infections
- Improves survival and HIV-related outcomes
- Improves quality of life

Determinants of health

- Health or ill health is the result of a combination of different factors.
- There are different perspectives in expressing the determinants of health of an individual or a community

Determinants ...

health field concept

- According to the “Health field” concept.
- There are four major determinants of health or ill health

1. Human Biology

- Every Human being is made of genes.
- In addition, there are factors, which are genetically transmitted from parents to offspring.
- As a result, there is a chance of transferring defective trait.
- The modern medicine does not have a significant role in these cases.

Determinants ...

- a. Genetic Counseling: For instance during marriage parents could be made aware of their genetic component in order to overcome some risks that could arise.
- b. Genetic Engineering: may have a role in cases like Breast cancer

Determinants ...

2. Environment:

- Those are factors **outside** the human body.
- They influence health include
 - a. Life support, food, water, air etc
 - b. Physical factors, climate, Rain fall
 - c. Biological factors: microorganisms, toxins, Biological waste,
 - d. Psycho-social and economic e.g. Crowding, income level, access to health care
 - e. Chemical factors: industrial wastes, agricultural wastes, air pollution, etc

Determinants ...

3. Life style (Behavior): is an action that has a specific frequency, duration, and purpose, whether conscious or unconscious.

- It is associated with practice.
- It is what we do and how we act
- Life style of individuals affects their health directly or indirectly .eg
 - Cigarette smoking
 - Unsafe sexual practice
 - Eating contaminated food

Determinants ...

4. Health care organization

- Health care organizations in terms of their resource in human power, equipments, money and so on determine the health of people
- concerned with
 - a. Availability of health service
 - b. Scarcity of Health Services leads to inefficient health service and resulting in poor quality of health status of people

Determinants ...

- c. Acceptability of the service by the community
- d. Accessibility : in terms of physical distance, finance etc
- e. Quality of care that mainly focuses on the comprehensiveness, continuity and integration of the health care

Introduction to health care system

Health care delivery system

- main function
 - increase the coverage
 - assure quality of care
 - promoting preventive and curative activities
- to achieve such objectives
 - ✓ adequate and motivated personnel,
 - ✓ availability of medical supplied and
 - ✓ sustainable financial resources are very important

health care system...

- The main purpose of health service is to give **comprehensive** and **integrated** primary health care (Health Extension Package) at the community level
- approach emphasize on the **preventive and promotive aspect** of health care without neglecting essential curative services

health care system...

- approach now being exercised through out the country is to focus on communicable diseases, common nutritional disorders, hygienic and environmental health.
- Maternal and child health care, immunization against child hood diseases, family planning and reproductive infections diseases including tuberculosis, malaria and the control of sexually transmitted diseases and HIV/AIDS are also critical area to address.

Group assignment

- Primary health international and national aspects
- Its principle and application

Chapter 2: Health Ethics

Ethics is a branch of **philosophy** that deals with distinctions between **right** and **wrong** – with the moral consequences of human actions.

The ethical **principles** that arise in health practice and research include:

- Informed consent

- Confidentiality

- Respect for human rights

- Scientific integrity

ETHICS...

- Medical ethics - (patient-centered)
- Public health ethics –
(community/population-centered)
- Research ethics - (subject-centered)

Confidentiality

- Is state of keeping or being kept secret or private.
- You must maintain adequate records of your work
- You should take steps to protect the identity of individuals, groups and others without the individual's express permission

Cont'd

- Confidentiality should be upheld and no information concerning the client or patient given away **without the permission** of the client
- The results of any tests must be kept absolutely confidential eg.HIV
- However, **shared confidentiality** is encouraged

Cont'd

- you must take all reasonable steps to communicate clearly the extent of confidentiality they are offering to clients
- Normally this should be made clear in the initial contracting

Cont'd

- Any agreement between the professional and the client or patient about confidentiality may be reviewed and changed by **joint negotiation**

Cont'd

- Counselors must not disclose any information about a client or patient to colleagues not directly involved in their care or their patients without first seeking consent of the client
- you must make provisions for maintaining confidentiality in the storage and disposal of client or patient records

Clients' Rights

- Confidentiality
- Privacy
- Refuse any activity
- Be treated with respect
- Information
- Ask Question

Values, Attitudes

Values

- Principles or standards that lead to judgments of either relative or absolute utility, goodness, or importance or that guide choices among alternate means or actions
- Values are relatively stable and resist changes

Attitudes

- A relatively enduring tendency to respond to someone or something in a way that reflects a **positive** or a negative evaluation of that person or thing
- Attitudes are **learned** predispositions or social orientations to act in accordance to the evaluation -favorably or unfavorably

Individual Rights and PH Ethical Issues

- Right to quality health services
- Provider responsibility to act for benefit of client
- Euthanasia - right to die
- Confidentiality – right to privacy
- Informed consent – right to know
- Birth control – religion vs. individual rights
- Supply and distribution of resources for health
- Incentives - disincentives
- Equity – social, ethnic, regional
- Social solidarity

Universal Declaration of Human Rights, 1948

Article 25.

Everyone has the right to a standard of living adequate for the health and well-being of himself and of his family, including food, clothing, housing and medical care and necessary social services etc.

Health for All

- WHO definition of health, 1948
- Alma Ata, 1978 Health for All
- Health care as a universal human right
- Government responsibility
- Wide acceptance
- Important to help shift priorities
- Still unfulfilled
- Health targets
- Priorities and cost-benefit decisions

Ethical Principles

- **Autonomy**
- **Beneficence**
- **Non maleficence**
- **Justice etc.**

1. Autonomy

- is the promotion of **independent** choice, self determination and freedom of action
- It means independence and ability to be self directed in healthcare.
- It is the basis for the client's right to self-**determination.**

Autonomy implies for basic elements

- autonomous person is
 - ✓ respected
 - ✓ be able to determine personal goals
 - ✓ has the capacity to decide on a plan of action
 - ✓ has the freedom to act upon the choices

2. Beneficence

- is doing or promoting **good**

3 principle of beneficence

- Promote good
- Prevent harm
- Remove evil or harm

3. Non maleficence

- is the converse of **beneficence**
- means to avoid doing harm
- is to avoid causing deliberate harm
 - ✓ E.g. Experimental research that have negative consequences on the client

4. Justice

- is fair, equitable and appropriate treatment
- **5. Veracity:** means telling the truth, which is essential to the integrity of the client-provider relationship
- **6. Fidelity:** being **faithful** to one's commitments and promises

7. Confidentiality

- “a secret or private matter not to be divulged to others”
- is the requirement of health professionals (HPs) to keep information obtained in the course of their work private

Patient's bill of rights

1. has a right **to considerate and respect full care.**
2. to obtain from his physician **complete current information concerning his diagnosis, treatment and prognosis**
3. has the right to receive from his physician information necessary to give informed consent prior to the start of any procedure and / or treatment

Patient's bill of rights...

- 4. right to refuse treatment to the extent permitted by Law and to be informed of the medical consequences of his action
- 5. right to every consideration of his privacy concerning his own medical care program
- 6. right to expect that all communications and records pertaining to his care should be treated as confidential

Patient's bill of rights...

7. right to expect that within its capacity a hospital must make reasonable response to the request of a patient for their services
8. right to obtain information as to any relationship of his hospital to other health care and educational institutions as far as his care is concerned
9. right to be advised if the hospital proposes to engage in or perform human experimentation affecting his care or treatment

Patient's bill of rights...

- 10. right to expect reasonable continuity of care
- 11. right to examine and receive an explanation of his bill regardless of the source of payment
- 12. right to know what hospital rules and regulations apply to his conduct as a patient

Chapter 3: Communication

INTRODUCTION

“Communication is an exchange of facts, ideas, opinions, or emotions by two or more persons”.
(W.H. Newman and C.F.

Summer Jr.)

Why? helps us better understand a person or situation and enables us to resolve differences and build trust and respect

Why Effective Communication?

- The ability to effectively communicate with others is one of the most important tools for professional and personal success.
- Effective communication can help you influence others.
- Your capacity to communicate is often seen as an indicator of your ability and intelligence.

Why communication? ...

- To have dialogue with communities.
- Influence decision makers to adopt health promoting policies and laws.
- Raise awareness among decision makers on issues regarding poverty, human rights, equity, environmental issues, etc...
- Ensure that the public gives support to government health promoting activities.
- Communicate new laws and policies to the public
- Raise public awareness in order to mobilize community participation.
- Develop community action on health issues.

- 80% of workplace issues are communication related

Types of Communication

1. One-way communication

- is a linear type of communication in which information flows from the source to the receiver
- There Is no input (feed back) from the receive

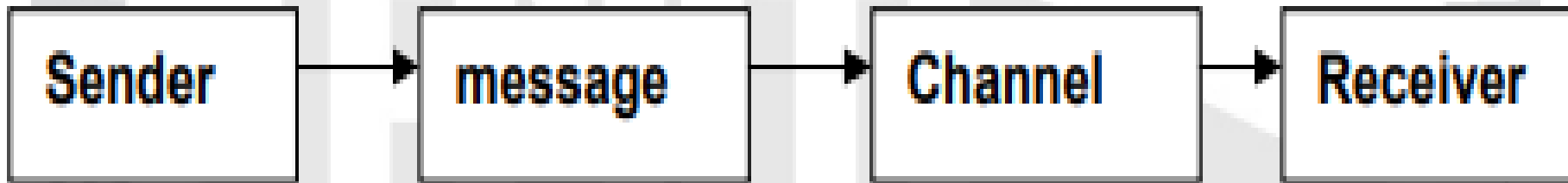
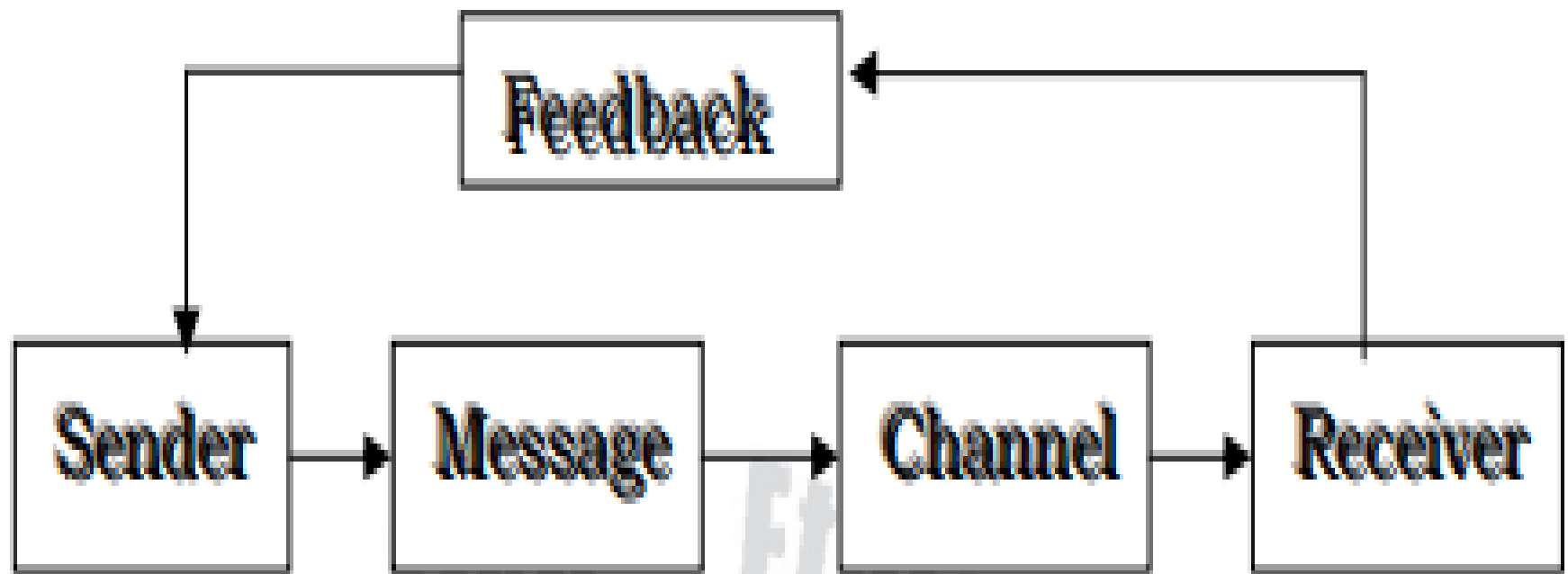


Fig.4.1. One way communication

Types of Communication...

2. Two-way communication

- As the message is more complex, two-way communication becomes essential.
- information flows from the source to the receiver and back from the receiver to the source.
- feedback allows the sender to find out how the message is being received and so it can be monitored and adapted to better suit the receiver's needs.



4.2 Complete Communication Model

Two way communication

Components of communication

1) Source (sender)

- Originator of message
- Can be from an individual or groups, an institution or organization.
- People are exposed to communication from different source but most likely to accept a communication from a person or organization that they trust i.e. has high source credibility.
- Depending on the community, trust and source credibility may come from: Personal qualities or actionse.g. a health worker who always comes out to help people at night.

Components...

- [?] Qualification and training
- [?] A person's natural position in the family or community, e.g. village chief or elder.
- [?] The extent to which the source shares characteristics such as culture, education, experiences with the receiver.

Components...

2) Message

- It consists of what is actually **communicated** including the actual appeals
- words, and pictures and sounds that you use to get the ideas across

A message is said to be good if it:

- Is Epidemiologically correct (evidence based)
- Is affordable (feasible)
- Requires minimum time/effort
- Is realistic
- Is culturally acceptable
- Meets a felt need
- Is easy to understand

Components...

3) Channel

- Is a physical means by which message travels from a source to a receiver
 - commonest types of channels are verbal, visual, printed materials or combined audio visual and printed materials

Components...

4) Receiver (Audience)

- person or a group for whom the communication is intended
- The first step in planning any communication is to consider the intended audience.

- Before communication, the following characteristics of audiences should be analyzed.
 - Educational factors: can they read? What type of appeals might convince them?
 - Sociocultural factors: What do they already believe and feel about the topic of communication?
 - Patterns of communication: how people show respect when talking to another person? What time of the day

Components...

5) Effect and feedback

- is the change in receiver's knowledge, attitude and practice or behavior.
- Feedback is the mechanism of assessing what has happened on the receiver after communication has occurred.

Communication stages

Stage 1: Reaching the intended audience

Stage 2: Attracting the audience's attention

Stage 3: Understanding the message
(perception)

Stage 4: Promoting change (acceptance)

Stage 5: Producing a change in behavior

Stage 6: Improvement in health

Common communication approaches

- Informing - The new idea is introduced and made familiar to the target audience.
- Educating - The new idea is explained including its strengths and weaknesses.
- Persuading- The audience is given **convincing** argument that motivates them to take an action or accept a new idea.
- Entertaining - The attention of the audience is drawn to the new idea by stimulating the audience's emotions

Methods of Communication

1. Intra - Personal communication(takes place inside a person)
2. Inter - Personal communication (interaction between two or more people who are together at the same time and place)
2. Mass communication(means of transmitting messages to a large audience that usually reaches a large segment of the population)

Advantages and limitations inter personal communication

Advantages

- Two way communication
- The communication could utilize multi channels (both verbal and non verbal) i.e. far more channels are involved than is possible in mass communication.
- Useful when the topic is a taboo or sensitive.

Advantages and limitations...

Limitations

- Requires language ability of the source.
- Requires personal status.
- Needs professional knowledge and preparation.

Mass communication

Advantage:

- Reach many people quickly
- They are believable specially when the source is a credible one

Limitation:

- One sided (linear)
- Doesn't differentiate the target

BARRIERS TO EFFECTIVE COMMUNICATION

- A breakdown can occur at any point in the communication process.
- Barriers (obstacles) can inhibit communication, resulting in misunderstanding, lack of response or motivation and distortion of the message

Common barriers to effective communication

- Competition for attention (noise)
- Language difference and vocabulary use
- Age difference
- Attitudes and Beliefs

Characteristics of effective communication

- All barriers have been removed.
- The proper media has been chosen.
- A good presentation has been made.
- Two – way communication has been established